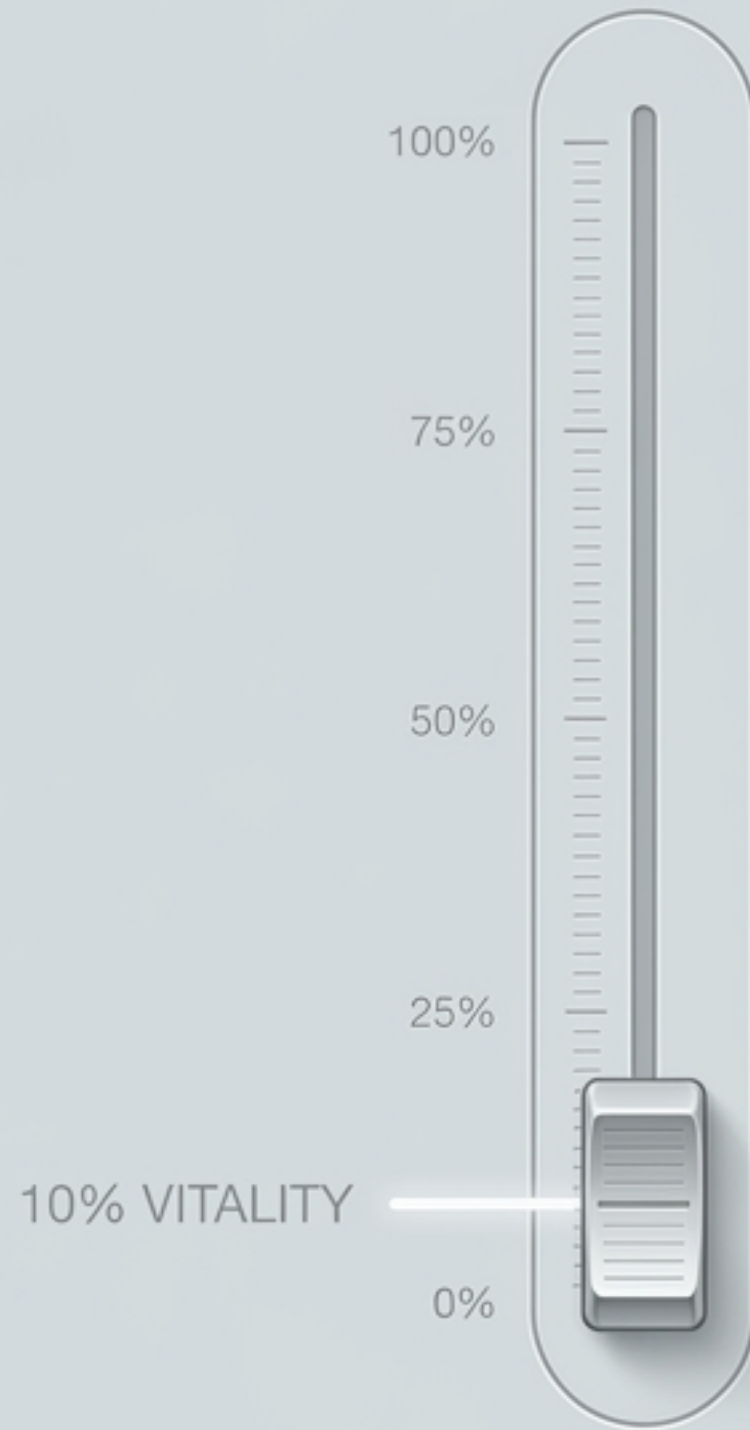


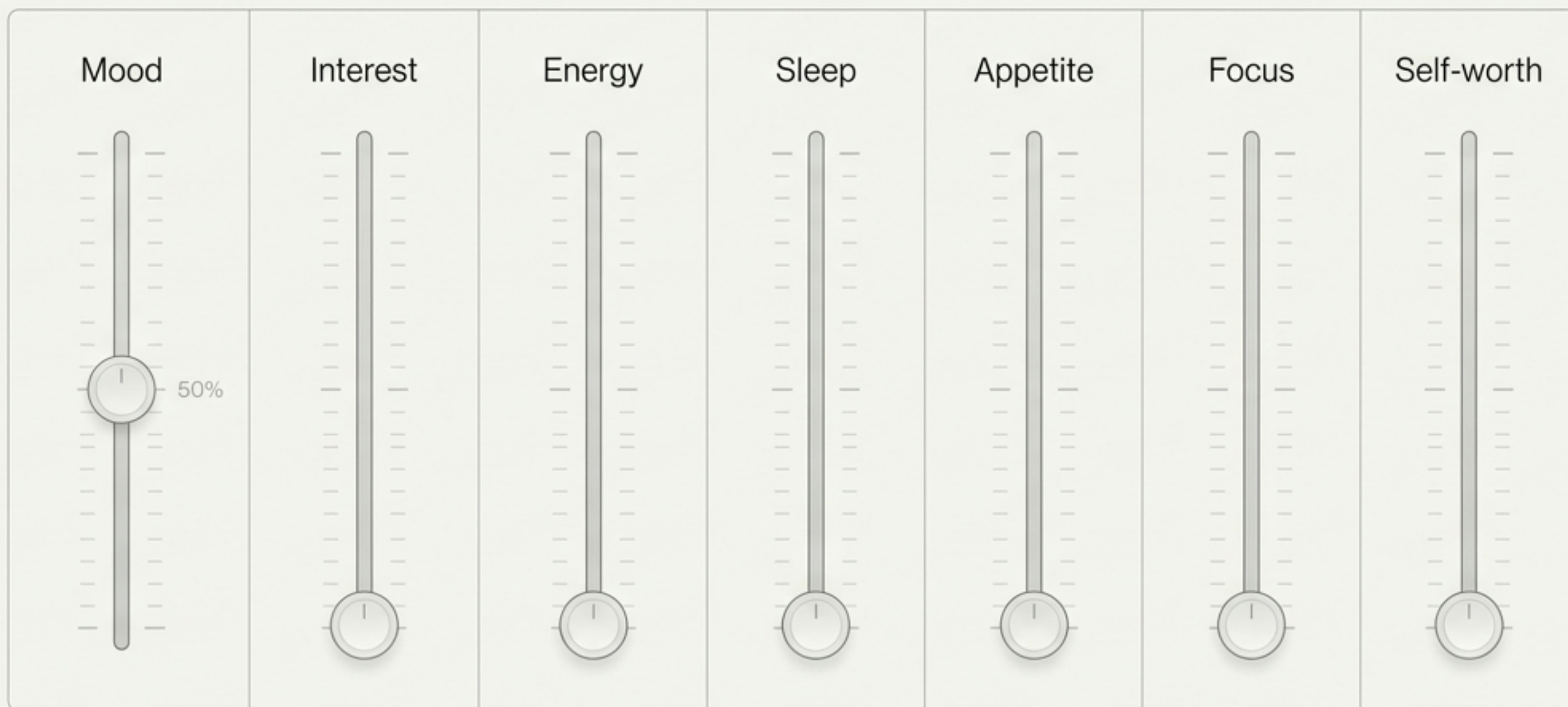


The Dimmer Switch

Understanding the physical state of depression, the truth about the serotonin story, and the mechanics of getting your vitality back.

Sadness is a feeling. Depression is a state.

Depression isn't extreme sadness, and it isn't the opposite of happiness. It is a systemic turning down of human function that flattens good feelings and bad ones alike.

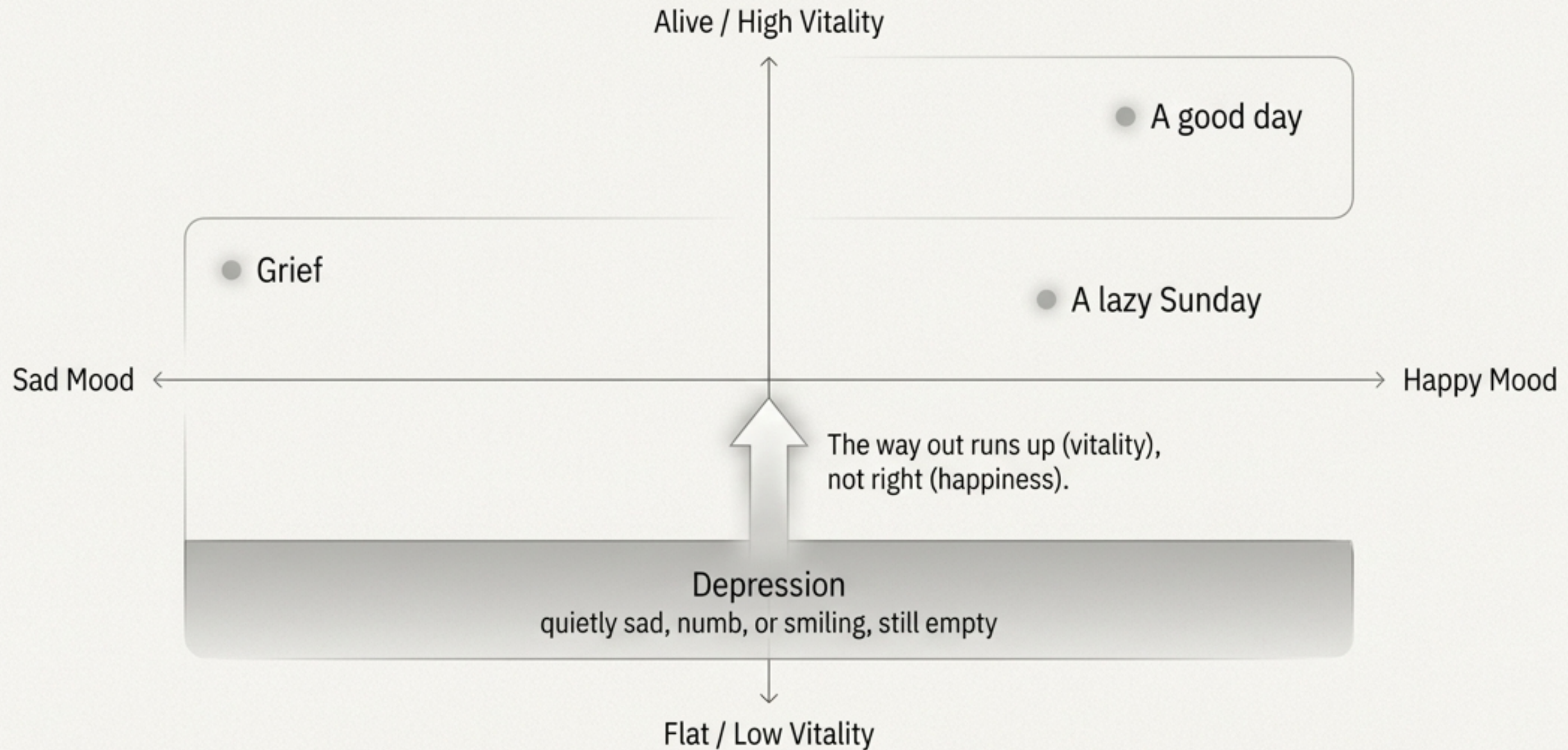


Distinguishing a natural emotion from a clinical state

Sadness		Depression
Cause: About something: a loss, a letdown, a goodbye		Can arrive with no clear reason, or outlast the one it had
Shape: Comes in waves, often set off by reminders		Flat and steady, most of the day, nearly every day
Other feelings	Still there. You can laugh in the middle of grief	Turned down too. Many people feel numb, empty or irritable rather than sad
Good things	Still land, even if briefly	Stop landing. Clinicians call this anhedonia
Self-worth	Mostly intact	Erodes into guilt, worthlessness, feeling like a burden
Time	Eases over days to weeks	Two weeks or more, and often doesn't lift on its own

Rule of Thumb: Grief can turn into depression, and the two can sit together. If the right-hand column has fit for two weeks or more, book a GP appointment.

Chasing happiness aims at the wrong map entirely



“The opposite of depression is not happiness, but vitality.” — Andrew Solomon

In a 2014 study, people who valued happiness most actually experienced more depressive symptoms.

Of the nine signs doctors ask about, only one is 'feeling down'

To be diagnosed via the PHQ-9, you need low mood or lost interest. Clinical depression is entirely possible without feeling sad at all.



If you are having thoughts of being better off dead or hurting yourself, please call Lifeline (13 11 14) or 000 today. You don't have to be sure it's serious enough.

How the low-serotonin story fractured

1950s: Accident

Two drugs, including a tuberculosis medication, lift mood by chance.

1987: Marketing

Prozac launches. "Chemical imbalance" becomes the standard public explanation, driven heavily by advertising.

2022: Review

A massive umbrella review at UCL pools decades of data and finds no convincing evidence that depression is caused by low serotonin.

1965: Hypothesis

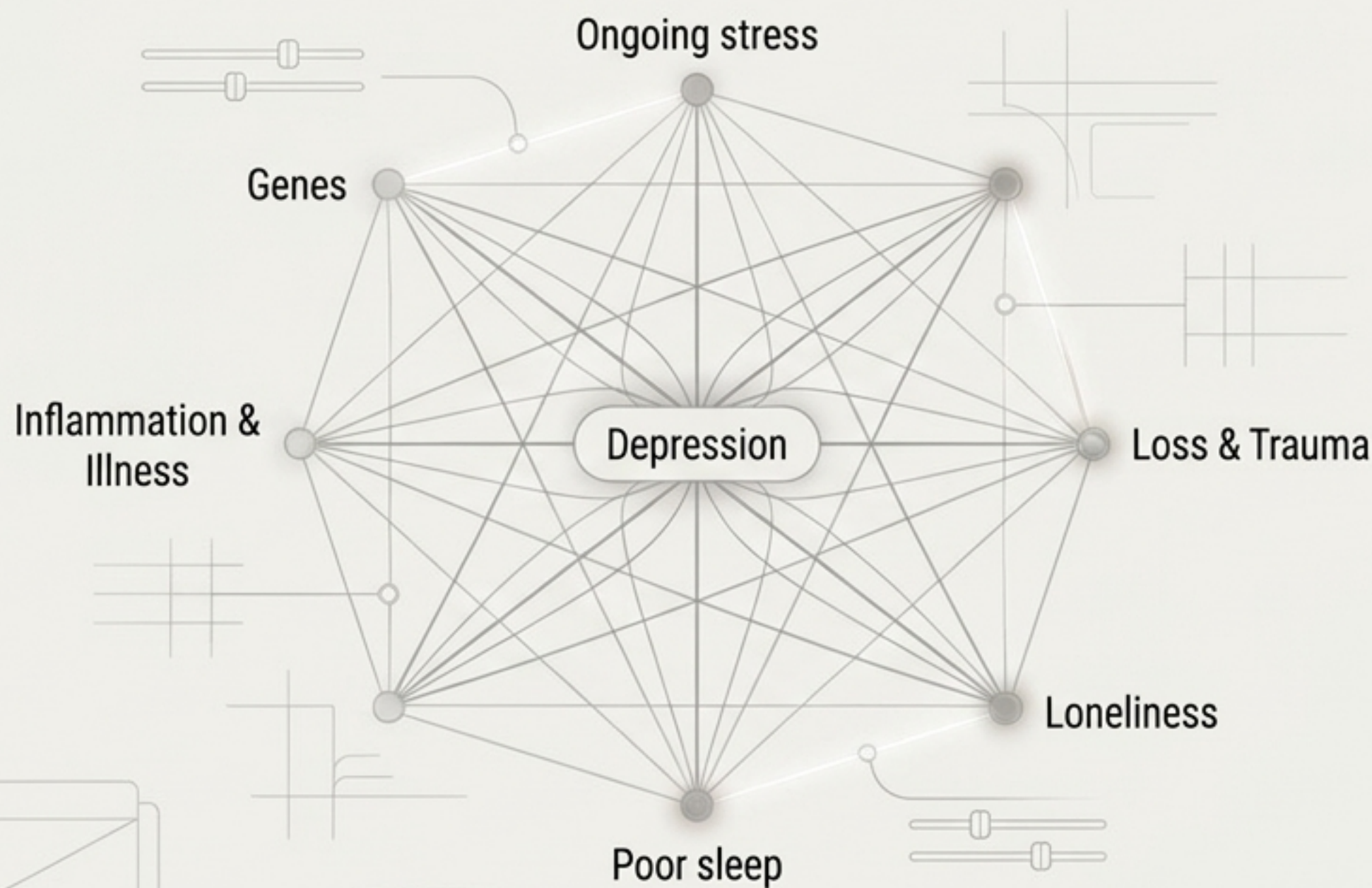
Reasoning backwards: If these drugs help, maybe depression is a shortage of brain chemicals.

2014: Side Effect

A study reveals this story hurts patients: telling people they have a chemical imbalance makes them pessimistic about recovery and strips them of agency.

The modern consensus: A web of interactions, not a single chemical

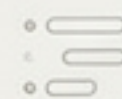
A one-chemical story was too simple, but the physical reality remains. Depression physically alters sleep, stress hormones, and how the brain learns from experience.



The Antidepressant Nuance



They still help: In a 2018 Lancet review of 116,000 adults, all 21 antidepressants beat placebos.



The mechanism: Serotonin rises in hours, but mood shifts take weeks. They work through slower, complex brain changes, not a simple "top-up".



Safety Note: Never stop suddenly; plan a gradual reduction with a prescriber to avoid withdrawal.

If the levers are complex, how do we pull them?

An explanation with no levers makes
recovery look impossible. But
nudged up ~~on~~ 1%.

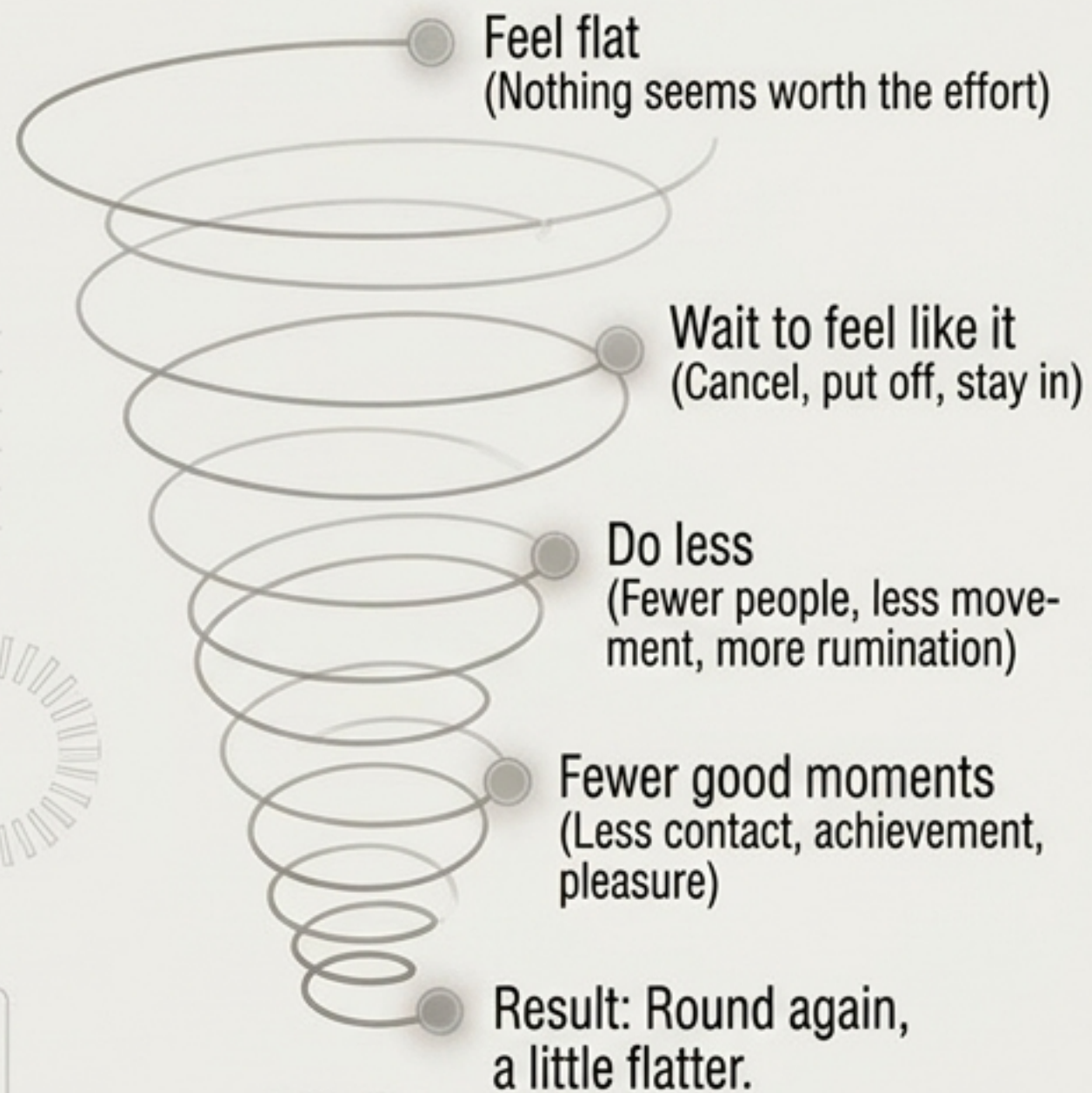


But because depression isn't a simple
chemical shortage, you actually have
multiple levers you can pull: sleep,
movement, connection, and purpose.

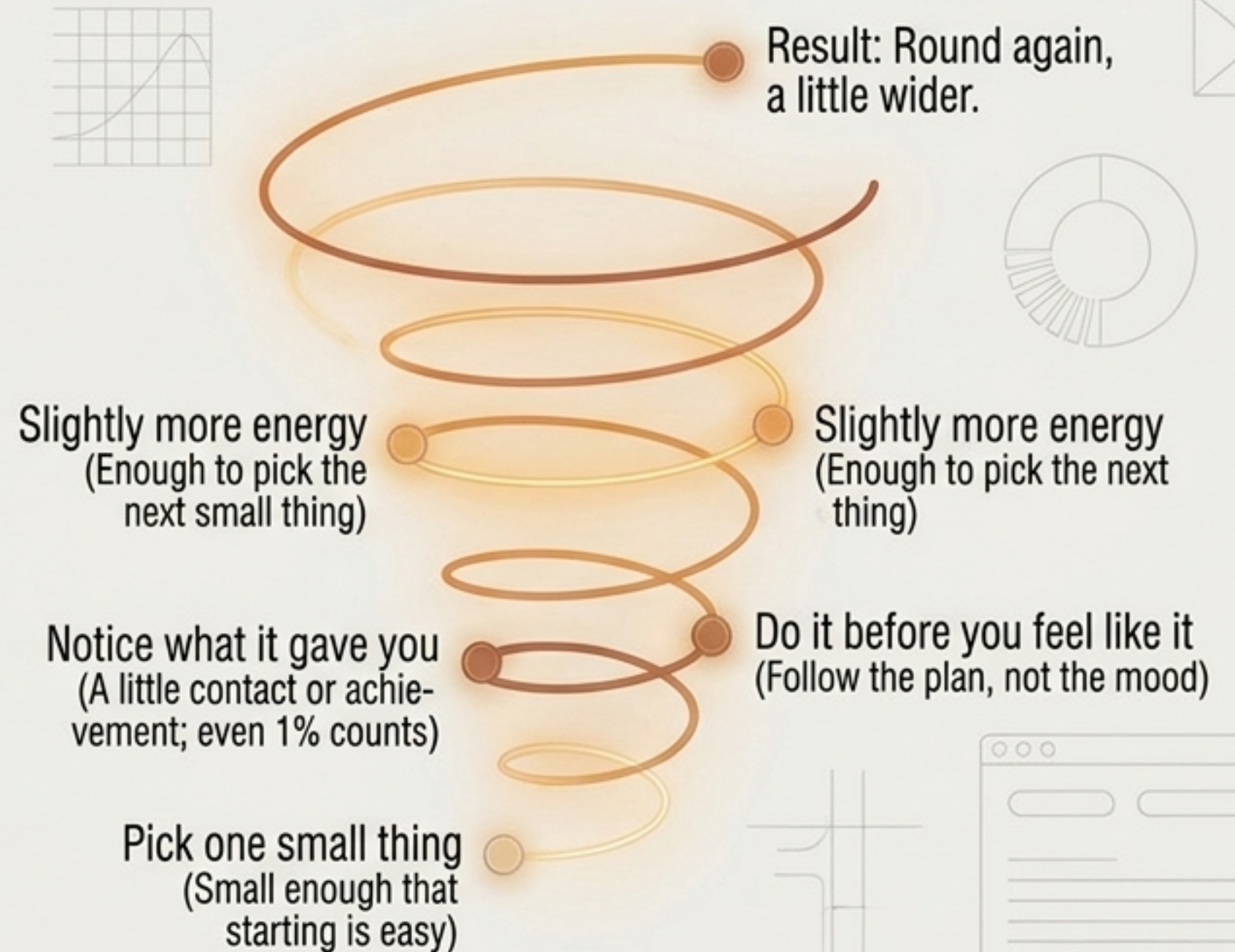
Depression tells you to wait until you feel like it. But the physics of recovery run in
reverse: **Motivation comes AFTER you start. Action precedes feeling.**

The opposing physics of motivation

The Shrinking Loop

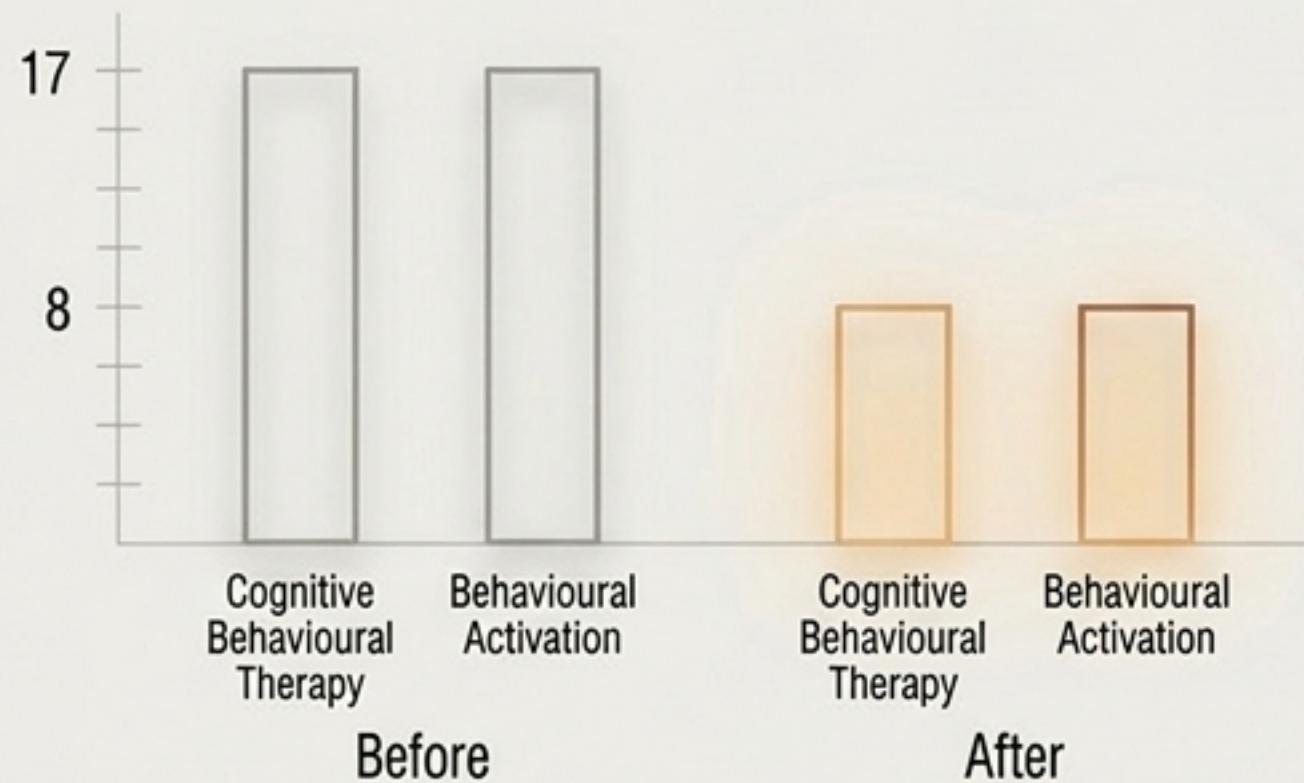


The Activation Loop



The empirical evidence for movement and action

Therapy - The COBRA Trial, Lancet 2016



Behavioural Activation (scheduling what matters) proved exactly as effective as full Cognitive Behavioural Therapy (CBT) for 440 adults. Furthermore, it **cost 21% less to deliver**.

Movement - BMJ Review, 2024



A review of 218 trials (14,170 people) found walking, yoga, and strength training all bring moderate drops in depression. The rule of thumb: **harder sessions help more than light ones**, and **yoga/strength** have the highest adherence.

The 4Cs: Ask what matters, not what you feel like

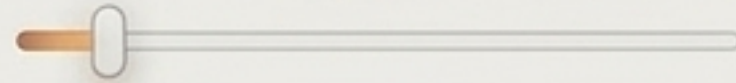
Depression flattens all four of these modes simultaneously. The goal is **one small move** per mode, done before the feeling arrives.



Chill

Sleep and Energy

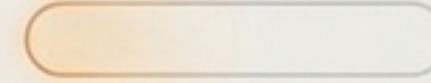
Steadying the rhythm.



Constructive

Starting and Finishing

Shrinking the task.



Caring

People and Self-Talk

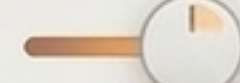
Letting one person in.



Curious

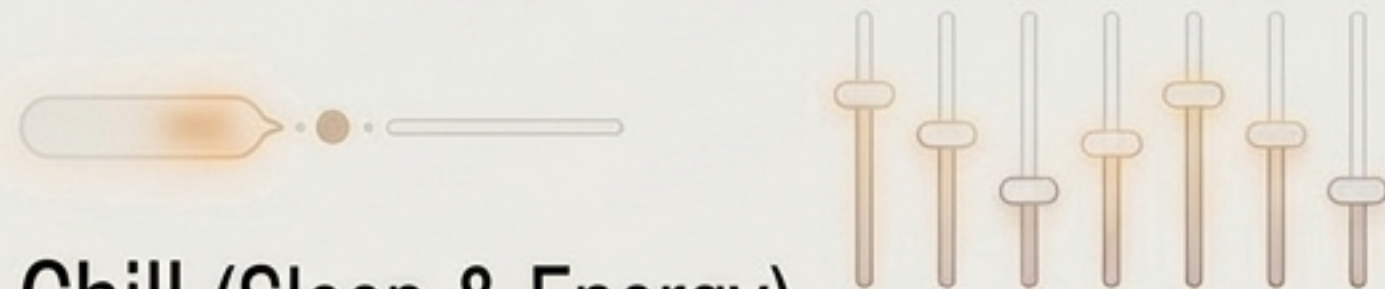
Interest and Values

Finding what still matters.



Note: Start with Chill and Constructive; rhythm and one finished thing make the other two easier.

Tactical Blueprint: Rhythm & Execution



Chill (Sleep & Energy)

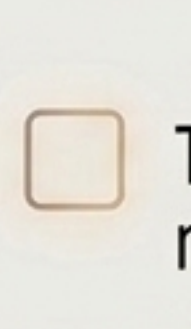
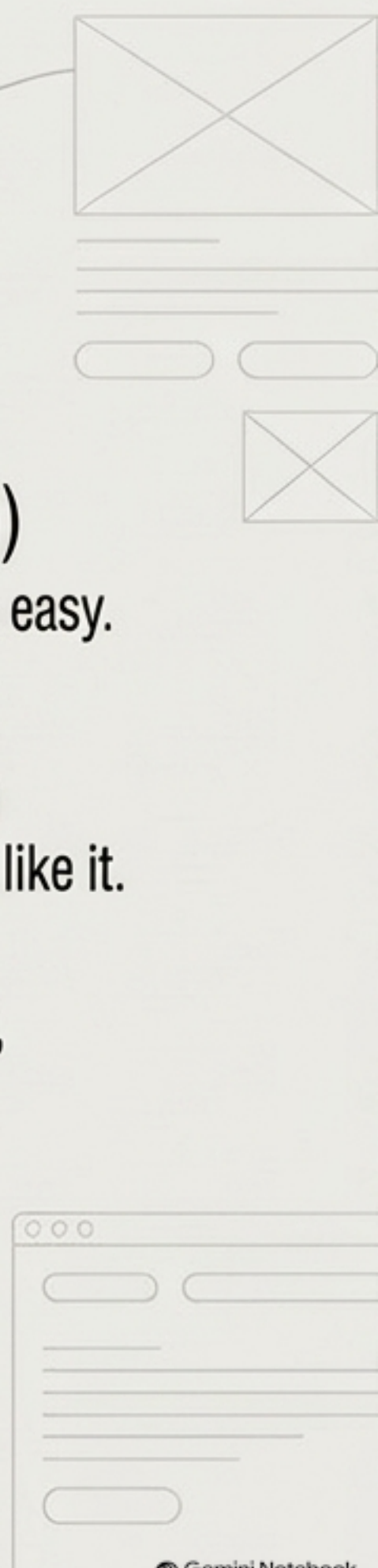
A steady rhythm comes before motivation, not after.

- 
- ☐ Today: Get outside in daylight for 10 minutes before midday, even if it's grey.
 - ☐ This Week: Fix your wake-up time, every day including weekends. Let bedtime follow it.
 - ☐ Keep Doing: Cut back on alcohol. It eases the evening but makes the next day heavier.
- 



Constructive (Starting & Finishing)

Every task looks huge. Shrink it until starting is easy.

- 
- ☐ Today: Pick one task you can finish in 10 minutes. Do it before deciding if you feel like it.
 - ☐ This Week: Move three times (brisk walk, yoga, strength). Harder counts for more.
 - ☐ Keep Doing: Each evening, choose tomorrow's one thing and write it where you'll see it.
- 

Tactical Blueprint: Connection & Interest

Caring (People & Self-talk)

Pulling away feels like rest, but it starves you of feedback and reasons to act.

- ☐ Today: Message one person today. It doesn't have to be about how you feel.
- ☐ This Week: Tell one person you trust how it's actually been, or book your GP.
- ☐ Keep Doing: When the harsh voice says "useless" or "burden," answer it exactly the way you'd answer a friend.

Curious (Interest & Values)

Look for 1% better, not "happy."

- ☐ Today: Rate your mood 0 to 10 before and after one activity. Notice what moves it, even slightly.
- ☐ This Week: Spend 15 minutes on something you used to care about, without needing to enjoy it.
- ☐ Keep Doing: List what matters to you (making, learning, people) and pick next week's small things from it.

The Safety Net: When to bypass the steps and get help

Small moves work best alongside proper care. If any of these are true, talk to a person, not a page.

Thinking about death, suicide, or hurting yourself.

Lifeline (13 11 14), Suicide Call Back (1300 659 467), or 000.

Low mood with voices or strange beliefs.

Needs assessment today (Emergency dept or GP).

Two weeks or more of life slipping (work, relationships).

Book GP. (GP can also check for physical causes like thyroid, anaemia, sleep apnoea).

You've also had highs (racing ideas, risky spending, low sleep need).

Tell GP before starting antidepressants (bipolar is treated differently).

Pregnant or new baby.

PANDA (1300 726 306).

Drinking/using more to cope.

National Alcohol/Drug Hotline (1800 250 015).

Getting care in Australia: Ask your GP for a Mental Health Treatment Plan (up to 10 Medicare-rebated sessions). Or call Medicare Mental Health Check In (1800 595 212) for free, no-referral support.

For the loved ones: Making the next step easier

You cannot lift depression for them. But you can lower the friction of recovery and keep showing up.

- ☐ **Skip 'cheer up':** It aims at happiness. Invite them to do one small thing with you instead (a walk, an errand).
- ☒ **Ask directly about suicide:** Asking doesn't put the idea in someone's head. It tells them they are allowed to say it out loud. (Call 13 11 14 together).
- ☐ **Make the first step smaller:** Offer to book the GP, drive them there, or just sit in the room while they make the call.
- ☒ **Keep inviting:** Expect to hear 'no' for a while. The invitation itself tells them they still belong.

Vitality comes back in steps. Keep the door open.